SACTCHECK TREATMENT ASSESSMENT

Bevacizumab + capecitabine

NCCP 00623 | 3a
SACTCheck v0.83.4

TUMOUR GROUP	INDICATION	COURSE
Gastrointestinal	**Metastatic or unresectable colorectal cancer when irinotecan- or oxaliplatin-combination chemotherapy is unsuitable.**	**Advanced disease | Until progression/toxicity | q21d**
TREATMENT CONTEXT	**ASSESSMENT ID**	**ASSESSMENT TIME**
Not entered	**JSON-DEMO-001**	**16/10/2026, 23:36**

ENCODED CRITERIA RESULT

1 encoded criterion requires clinical review

Review the highlighted protocol criteria and the current NCCP source before the final treatment decision.

ENTERED VALUES AND ENCODED CRITERIA

Standard assessment

DOMAIN	ENTERED VALUE	ENCODED CRITERION	RESULT
Creatinine clearance category	30-50 mL/min	>= 30	**Additional clinical review/context**
Creatinine clearance category	30-50 mL/min	<= 50	**Additional clinical review/context**
Haemodialysis	No	= Yes	**Criterion met**

COMPONENT-SPECIFIC PROTOCOL ACTIONS

Protocol-level guidance; verify prescribed dose/current level

Capecitabine: Clinical review
Source discrepancy: NCCP 00623 table 2 gives 100% at CrCl >=30. Capecitabine SmPC section 4.2 gives 75% for baseline CrCl 30-50 with a 1250 mg/m^2 starting dose, but no reduction for a 1000 mg/m^2 start. Confirm the actual starting dose and resolve with consultant/pharmacy; no automatic percentage is selected. page 4 | Table 2

UNASSESSED DOMAINS

ECOG performance status, Hypersensitivity to capecitabine or its excipients, Previous severe, unexpected fluoropyrimidine reaction, Brivudine now or within the previous four weeks, Pregnant, Breastfeeding, DPD status, Assessment timing (optional) + 45 more

Blank domains were not assumed normal.

CLINICIAN DECISION

Not documented
Not documented

CLINICAL DECISION SUPPORT - NOT TREATMENT CLEARANCE

SACTCheck applies encoded NCCP protocol criteria to clinician-entered information. This output does not constitute treatment clearance and does not replace complete clinical assessment, the current official NCCP protocol, prescribing information, local policy or professional judgement. The responsible oncology clinician retains responsibility for the final treatment decision.